


Laboratory Investigation Report

Patient Name	: Mr. Vineet	Centre	: 5054 - Motherland Hospital
Age/Gender	: 22 Y 6 M 21 D / M	OP/IP No/UHID	: OP/1019956/MLH/24/011848
MaxID/Lab ID	: ML05009859/4337032601346	Collection Date/Time	: 31/Mar/2026 10:45AM
Ref Doctor	: Dr.Mukesh Chandra Jha	Reporting Date/Time	: 31/Mar/2026 01:16PM

Clinical Biochemistry


SIN No: B2B9465983

SGOT - Aspartate Amino Transferase, Serum*

Date	31/Mar/2026	Unit	Bio Ref
	10:45AM		Interval
SGOT- Aspartate Transaminase (AST) UV without P5P	26.0	U/L	< 50

Interpretation

Increased in acute hepatitis especially viral hepatitis, liver cell necrosis or injury of any cause, including cholestatic or obstructive jaundice, chronic hepatitis and drug induced injury to liver.

CRP- C- Reactive Protein*, Serum

Date	31/Mar/2026	Unit	Bio Ref
	10:45AM		Interval
CRP Immunoturbidimetric	50.0		

Interpretation This helps in detecting neonatal septicemia, meningitis and useful to assess the activity of inflammatory diseases like rheumatoid arthritis. It is increased after myocardial infarction, stress, trauma, infection, inflammation, surgery, or neoplastic proliferation. The increase with inflammation occurs within 6 -12 hours and peaks at about 48 hours.

Ref Range :

Mg/L	Mg/dL
< 5.0	< 0.5

SGPT - Alanine Amino Transferase, Serum*

Date	31/Mar/2026	Unit	Bio Ref
	10:45AM		Interval
SGPT- Alanine Transaminase (ALT) Kinetic Rate using LDH	38.0	U/L	17 - 63

Interpretation

Increased in Acute Liver Cell necrosis of any cause, severe shock right heart failure, acute anoxia (e.g. status asthmaticus), extensive trauma and left heart failure.

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist

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Test Name	Clinical Pathology Result	Unit	Bio Ref Interval
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Urine Routine And Microscopy*
Macroscopy

Colour Visual Observation/ Automated	Pale Yellow		Pale Yellow
PH Double Indicator	6.5	..	5-6
Specific Gravity pKa change	1.015		1.015 - 1.025
Protein Protein-error of indicators	Trace		Nil
Glucose. Enzyme Reaction	Negative		Nil
Ketones Acetoacetic Reaction	ABSENT		Nil
Blood Benzidine Reaction	ABSENT		Nil
Leukocyte Esterase Leukocyte esterase reaction	Negative		Nil
Bilirubin Diazo Reaction	Negative		Nil
Urobilinogen Ehrlichs Reaction	Normal		Normal
Nitrite Conversion of Nitrate	Negative		

Microscopy

Red Blood Cells (RBC) Light Microscopy/Image capture microscopy	Nil	/HPF	Nil
White Blood Cells Light Microscopy/Image capture microscopy	3-5	/HPF	0.0-5.0
Epithelial Cells Light Microscopy/Image capture microscopy	2-4	/HPF	0.0 - 5.0
Cast Light Microscopy/Image capture microscopy	Nil	/LPF	Nil
Crystals Light Microscopy/Image capture microscopy	Nil	..	Nil

Kindly correlate with clinical findings

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Clinical Pathology


SIN No: B2B9465983

Test Name	Result	Unit	Bio Ref Interval
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Dr. Mrinalini Priyadarshini
 DCP, DNB (Path.)
 Consultant Pathologist


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Hematology


SIN No: B2B9465983

Test Name	Result	Unit	Bio Ref Interval
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Malaria Antigen – P Vivax & P Falciparum*, EDTA

Malaria Antigen	Negative		Negative
Immunochromatography - pLDH & HRP2			

Interpretation Rapid card test for malaria is a combo kit designed to test Plasmodium falciparum and Plasmodium vivax species of malaria. This is a combo kit coated with specific monoclonal antibodies against pLDH of the P. Vivax and HRP2 of the P. Falciparum. This kit can also detect the combined infection by these two species.

The result of this test needs to be corroborated with clinical features and other laboratory findings. Positive result with faint test line or false negative may be seen in low parasite density. Negative result can also be seen in prozone effect – i.e. very high antigen concentration compared to antibody concentration.

False positive result may be seen in acute Schistosomiasis.

Test may remain positive even after successful anti-malarial therapy and therefore should not be used for monitoring response to anti-malarial treatment.

Advice: “Peripheral smear for Malarial Parasite”


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Hematology


SIN No: B2B9465983

ESR ,EDTA*

Modified (Westergren)

Date	31/Mar/2026 10:45AM	19/Aug/25 10:33AM	10/Sep/24 11:32AM	Unit	Bio Ref Interval
ESR (Modified Westergren)	22	12	25	mm/hr	<=10
Modified (Westergren)					

Interpretation

(Syn: Erythrocyte Sedimentation Rate)

ESR is measured as red cells fall through a column of blood. It is a sensitive index of plasma protein change. It can be affected by age, sex, menstrual cycle, pregnancy and drugs(e.g. OCP, steroids).

No fasting sample is required for ESR.

ESR is performed for the diagnostic purpose for temporal arteritis and polymyalgia rheumatica. It is also used for chronic inflammation.

High ESR is seen in - inflammatory disorders (e.g. infection , rheumatoid disease, tuberculosis), presence of paraproteinemia (e.g. multiple myeloma, lymphoma) and anaemia.

Low ESR is seen in - polycythemia, hypofibrinogemia, poikilocytosis, spherocytosis and sickle cell anaemia.

Normal ESR does not exclude organic disease.


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Hematology


SIN No: B2B9465983

CBC (Complete Blood Count)*, EDTA

Date	31/Mar/2026 10:45AM	19/Aug/25 10:33AM	11/Sep/24 12:10PM	10/Sep/24 11:32AM	Unit	Bio Ref Interval
Haemoglobin	16.44	17.13	15.60	16.34	g/dl	13.0 - 17.0
Modified cyanmethemoglobin						
Packed Cell, Volume	48.7	49.7	44.6	48.1	%	40-50
Calculated						
Total Leucocyte Count (TLC)	14.54	7.62	3.33	3.34	10~9/L	4.0-10.0
Electrical Impedance						
RBC Count	5.44	5.84	5.12	5.47	10~12/L	4.5-5.5
Electrical Impedance						
MCV	89.5	85.1	87.1	88.0	fL	83-101
Electrical Impedance						
MCH	30.2	29.3	30.5	29.9	pg	27-32
Calculated						
MCHC	33.8	34.5	35.0	34.0	g/dl	31.5-34.5
Calculated						
Platelet Count	215	187	125	120	10~9/L	150-410
Electrical Impedance						
MPV	9.90	11.91	12.57	11.98	fl	7.8-11.2
Calculated						
RDW	14.2	14.4	14.6	14.5	%	11.5-14.5
Derived from RBC Histogram						

Differential Cell Count

VCS / Light Microscopy

Neutrophils	76	65	52.07	65.70	%	40-80
Lymphocytes	17	20	30.58	21.69	%	20-40
Monocytes	06	13	16.41	11.39	%	2-10
Eosinophils	01	02	0.72	1.13	%	1-6
Basophils	00	00	0.22	0.09	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	11.05	4.95	1.73	2.19	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.5	1.5	1.0	0.7	10~9/L	1.0-3.0
Absolute Monocyte Count	0.87	0.99	0.55	0.38	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.15	0.15	0.02	0.04	10~9/L	0.02-0.5

Kindly correlate with clinical findings

***** End Of Report *****

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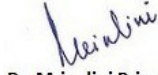
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Serology


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Test Name	Result	Unit	Bio Ref Interval
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Typhi Dot Test (IgM & IgG)*, Serum

Immunochromatography

Typhidot(IgG) Negative

Immunochromatography

Typhidot(IgM) Negative

Immunochromatography

Interpretation

- This is rapid card test, based on lateral flow chromatographic immunoassay.
- This is a screening test and definite clinical diagnosis should not be based on this single test result.
- The result is to be confirmed by other supplemental tests like blood culture and widal test.
- Positive result (IgM response) can vary according to time elapsed from the onset of fever and immunocompetence status.
- A negative result does not rule out recent or current infection. If S.typhi infection is still suspected, a repeat sample is advised after 5-7 days.
- False positive result can be seen in patients having high titer of rheumatoid factor.

Advise:

- First week of fever: Blood culture
- Second week of fever: Widal Tube test

Kindly correlate with clinical findings

***** End Of Report *****

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Results to follow:

Peripheral Smear Examination : Sample not yet received,Dengue NS 1 Antigen Test : Sample not yet received,Urine Culture & Sensitivity : Sample not yet received

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